Global action plan on physical activity 2018–2030: more active people for a healthier world

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economically, levels of inactivity increase. In some countries, levels of inactivity can be as high as 70%, due to changing patterns of transportation, increased use of technology and urbanization.

Physical activity levels are also influenced by cultural values. In most countries, girls, women, older adults, underprivileged groups, and people with disabilities and chronic diseases, all have fewer opportunities to access safe, affordable and appropriate programmes and places in which to be physically active.

The global cost of physical inactivity is estimated to be INT$ 54 billion per year in direct health care, in 2013, with an additional INT$ 14 billion attributable to lost productivity. Inactivity accounts for 1–3% of national health care costs, although this excludes costs associated with mental health and musculoskeletal conditions.

Multiple opportunities: multiple benefits

Physical activity can and should be integrated into the settings in which people live, work and play. Walking and cycling are key means of transportation and enable engagement in regular physical activity on a daily basis, but their role and popularity is declining in many countries. Sport and active recreation can help promote physical activity for people of all ages and abilities. Globally it can be a key driver of tourism, employment and infrastructure, and can also help in humanitarian programmes, fostering community development and social integration.

Physical activity is important across all ages. Active play and recreation is important for early childhood as well as for healthy growth and development in children and adolescents. Quality physical education and supportive school environments can provide physical and health literacy for long-lasting healthy, active lifestyles.

It is also important that adults can be physically active and less sedentary at work. Whether working or not, older adults, in particular, can benefit from regular physical activity to maintain physical, mental and social health and enable healthy ageing. Primary and secondary health and social care providers can help individuals of all ages become more active and prevent NCDs, while also using physical activity as a means to increase rates of rehabilitation and recovery.

Across all settings there are opportunities for digital innovations to promote and support people of all ages to be more active and to build upon the rapidly growing practice of mHealth to harness the potential of data to help promote, support and monitor physical activity.

Physical activity and the Sustainable Development Goals 2030

Investing in policies to promote walking, cycling, sport, active recreation and play can contribute directly to achieving many of the 2030 Sustainable Development Goals (SDGs). Policy actions on physical activity have multiplicative health, social and economic benefits, and will directly contribute to achieving SDG3 (good health and well-being), as well as other Goals including SDG2 (ending all forms of malnutrition); SDG4 (quality education); SDG5 (gender equality); SDG8 (decent work and economic growth), SDG9 (industry, innovation and infrastructure);

Mandate

In 2013, the World Health Assembly endorsed a global action plan on the prevention and control of NCDs *(6)*, and agreed a set of nine global voluntary targets, which include a 25% reduction of premature mortality from NCDs and a 10% relative reduction in the prevalence of insufficient physical activity by 2025 *(7)*. A recent review of global progress towards these targets concluded that progress has been slow and uneven across high-, middle-, and low-income countries *(8)*. Although the 2013 NCD global action plan provided World Health Organization (WHO) Member States with a set of broad policy recommendations to increase physical activity *(9)*, implementation and engagement with the necessary sectors outside of health has remained a significant challenge to progress in most countries.

At its 140th session in 2017, the WHO Executive Board agreed to endorse a proposal for the Secretariat to prepare an action plan on physical activity to be submitted for consideration by the Board at its 142nd session. It was requested to build upon existing NCD *(6)* and physical activity strategies *(10, 11)*, guidelines *(1)*, policy recommendations *(12)* and other relevant commitments endorsed by the World Health Assembly, and to link with the Sustainable Development Goals (SDGs) set for 2030. Accordingly, this Global Action Plan on Physical Activity 2018–2030 (hereafter referred to as the "action plan" or "global action plan") provides a framework for action, and proposes a set of specific policy actions to guide Member States to accelerate and scale activities towards achieving increased levels of physical activity. It also acknowledges the requests of Member States for stronger global, regional and national coordination, and the need for a whole-of-society paradigm shift in respect to both supporting and valuing all people being regularly active, according to ability and across the life course.

Development process

The global action plan was developed through a worldwide consultation process and involved establishing a WHO internal steering committee comprising multiple departments across relevant clusters and representatives from WHO Regional Offices, as well as with guidance from a multisectoral and a multidisciplinary global expert advisory group which met during July 2017. Following publication of Draft 1 of the action plan, six regional consultations were conducted with Member States, as well as eight public webinars, information sessions with United Nations agencies and permanent missions, awareness-raising through social and professional society media and a seven-week period of open online public consultation.

The process engaged with 83 Member States (including representatives from ministries of health, education, sports, transport and planning) as well as international sports associations, health and sport medicine organizations, institutes of public health, civil society and professional organizations across health, transport, urban planning and sports, the research and academic community, and the private sector. A total of 125 written submissions were received from interested stakeholders. Input from all the consultation processes informed the drafting and preparation of Draft 2. Following discussion of Draft 2 at the 142nd session of the Executive Board, comments and suggestions informed the development of this final action plan.

Although achieving the recommended levels of physical activity for optimal health for the general population carries low levels of risk, there are higher risks associated with participation in certain types of physical activity and for some subpopulation groups. Notable activities with higher risk of injury include contact sports (such as rugby, ice hockey) and walking and cycling, where road safety and or personal violence may present higher risks in some contexts. WHO recommendations on physical activity provide guidance on risk reduction *(1)* and this action plan recommends policy actions to address the wider socioenvironmental risks such as road safety for cycling.

Current levels of physical inactivity

The most recent available global comparative estimates from 2010 indicate that worldwide, 23% of adults and 81% of adolescents (aged 11–17 years) do not meet the WHO global recommendations on physical activity for health[1] *(6)*. Notably, the prevalence of inactivity varies considerably within and between countries, and can be as high as 80% in some adult subpopulations. Physical inactivity in adults is highest in the Eastern Mediterranean, the Americas, Europe and Western Pacific regions, and is lowest in the South-East Asia Region *(15)*. These rates increase with economic development, owing to the influence of changing patterns of transportation, use of technology, urbanization and cultural values *(16)*.

> Globally, 23% of adults and 81% of adolescents (aged 11–17 years) do not meet the WHO global recommendations on physical activity for health.[2]

Differences in levels of physical activity are also explained by significant inequities in the opportunities for physical activity by gender and social position, within as well as between countries *(15)*. Girls, women, older adults, people of low socioeconomic position, people with disabilities and chronic diseases, marginalized populations, indigenous people and the inhabitants of rural communities often have less access to safe, accessible, affordable and appropriate spaces and places in which to be physically active. Addressing these disparities in participation is a policy priority and underlying principle of this global action plan.

Updated global comparable estimates on physical inactivity in adolescents and adults are being prepared by WHO and will be available in 2018.[2]

[1] The WHO global recommendation on physical activity for health for adults is 150 minutes of moderate-intensity activity (or equivalent) per week, measured as a composite of physical activity undertaken across multiple domains: for work (paid and unpaid, including domestic work); for travel (walking and cycling); and for recreation (including sports). For adolescents, the recommendation is 60 minutes of moderate- to vigorous-intensity activity daily.
[2] These data are from 2010 *(6)*. Updated estimates will be forthcoming in 2018 in the document "WHO country comparable estimates on physical inactivity, 2016".

and prioritize access by pedestrians, cyclists and users of public transport to destinations and services, particularly educational, public open and green and "blue" spaces,[1] sports and leisure facilities, can reduce use of personal motorized transportation, carbon emissions, traffic congestion as well as health-care costs *(20)*, whilst also boosting the micro-economies in local neighbourhoods and improving health, community well-being and quality of life *(21, 22)*. Given the increasingly urbanized world, with more than 70% of the global population living in urban centres, cities have a particular responsibility and opportunity to contribute to this agenda through improving urban design and sustainable transport systems *(23)*.[2]

Sport is an underutilized yet important contributor to physical activity for people of all ages, in addition to providing significant social, cultural and economic benefits to communities and nations *(24, 25)*.[3] While sport can be a catalyst and inspiration for participation in physical activity *(26)*, the sports sector is also a significant employer and a key driver of tourism and infrastructure globally. Sport and active recreation can also contribute in emergency and crisis situations as part of humanitarian programmes aimed at health and social needs, as well as community development and integration *(27)*. Strengthening access to, and the promotion of participation in, sports and active recreation, across all ages and abilities, is an important element of increasing population levels of physical activity.

The General Conference of UNESCO observed in the International Charter of Physical Education, Physical Activity and Sport that "the practice of physical activity and sports is a fundamental right for all" *(28)* and the 2017 Kazan Action Plan endorsed at MINEPS VI *(29)* as well as the Commission on Ending Childhood Obesity *(30)*, identified active play and recreation as important elements of healthy growth and development in children, including those aged less than 5 years. Further, provision of quality physical education and supportive school environments can impart physical and health literacy for lifelong healthy, active lifestyles, prevention of NCDs and mental health disorders as well as strengthen academic outcomes. School-based policy initiatives are an essential component of endeavors to create a more active society.

Physical activity is important across all ages, and should be integrated into multiple settings.

Physical activity is important across all ages, and should be integrated into multiple daily settings. For many adults, the workplace is a key setting to be physically active and reduce sedentary behaviour. The trip to and from work, activity breaks, workplace programmes and incidental activity all offer opportunities for increased physical activity throughout the working day, and can contribute to increased productivity and reduction in injuries and absenteeism *(31)*. Whether working or not, older adults, in particular, can benefit from regular physical activity to maintain physical, social and mental health (including prevention or

[1] "Blue space" refers to space near rivers, lakes and oceans.
[2] Such as Partnerships for Healthy Cities (https://partnershipforhealthycities.bloomberg.org/, accessed April 2018).
[3] See: The Association for Sports for All (http://www.tafisa.org/, accessed April 2018).

The policy responses proposed in the global action plan are not only interconnected with achieving the SDGs, but also intersect with, and complement achieving, the goals and ambitions of other closely related strategies and plans endorsed by the World Health Assembly, including:

- **Commission on Ending Childhood Obesity *(30)***
- **Global Strategy and Action Plan on Ageing and Health 2016–2020 *(32)***
- **Global Plan for the Decade of Action for Road Safety 2011–2020 *(37)***
- **WHO Public Health & Environment Global Strategy *(38)***
- **The New Urban Agenda *(39)***
- **Mental Health Action Plan 2013–2020 *(40)***
- **Global Action Plan on the Public Response to Dementia 2017–2025 *(41)***
- **Global Strategy for Women's, Children and Adolescents' Health 2016–2030 *(42)***
- **Every Newborn Action Plan to End Preventable Deaths 2014 *(43)***
- **WHO Global Disability Action Plan 2014–2021 *(44)***
- **Global Nutrition Report 2017: Nourishing the SDGs *(45)***
- **United Nations Decade of Action on Nutrition 2016-2025 *(46)***

Moving forward – scaling national action

Given the diversity of ways to be active, and the multiple settings in which it is possible to increase participation, there are multiple policy opportunities across different sectors. National policy responses must address the multiple factors which determine participation; some of these are individual characteristics, knowledge and personal preferences, while others are related to the wider sociocultural contexts, such as family context, societal values, traditions, and economic and physical environments *(47)*. These, so called, "upstream" determinants of physical activity shape the equity of opportunities for participation and can further contribute to reducing inequalities in physical activity, health status and well-being *(48)*.

Effective national responses must include actions to address factors that impinge on the opportunities and abilities of all people to be active, as well as policy actions to protect and enhance those factors that enable and encourage participation. Accordingly, "upstream" population-based policy approaches to promote physical activity must be prioritized and interlinked with policy actions focused on "downstream" individually-centred interventions.

Effective implementation will require Member States to select a strategic combination of the recommended policy responses provided in this action plan, adapted and executed at national scale according to country context, cognizant of different needs and abilities of subpopulations.

However, despite strong evidence on effective solutions *(12)*, progress will remain an aspiration unless reliable dedicated resources, both human and fiscal, are secured to support promoting physical activity as a priority within NCD treatment and prevention, as well as establishing the strategic connections between key government departments, stakeholders and related policy priorities to enable sustained implementation at national and subnational levels.